

Patient Name: SARAH JOHNSHAN

DOB: 11/09/1971

DOS:9/18/2024

Gender : FEMALE

ACTIVE PROBLEMS:

- I10 - Essential Hypertension.
- F41.1 - Generalized Anxiety Disorder.
- K21.9 - GERD.
- R73.9 - Hyperglycemia.
- E78.5 - Hyperlipidemia.
- F33.42 - Major Recurrent Depressive Episode, in Full Remission.
- M19.90 - Osteoarthritis.

CHIEF COMPLAINT:

- The Chief Complaint is: Follow-up for depression and HTN.
- Patient is here for follow-up of the chronic conditions.
- Allergy list reviewed.
- Problem list reviewed.
- Medication reconciliation performed.
- Medication list reviewed.
- Taking medications as prescribed.
- No new symptoms.
- Essential hypertension.

Current Medication:

- Cyclobenzaprine HCl 5 MG Oral Tablet: Take 1 tablet at bedtime as needed for muscle spasms. Prescription valid for 90 days with 3 refills.
- Diclofenac Sodium 75 MG Oral Tablet Delayed Release: Take one tablet twice daily for pain and inflammation. Prescription valid for 90 days with 3 refills.
- Lovastatin 20 MG Oral Tablet: Take 1 tablet daily for cholesterol. Prescription valid for 90 days with 3refills.
- Sertraline HCl 50 MG Oral Tablet: Take 1 tablet every day for depression. Prescription valid for 90 days with 3 refills.
- Valsartan-hydrochlorothiazide 320-25 MG Oral Tablet: Take 1 tablet daily for blood pressure. Prescription valid for 90 days with 3 refills.

Patient: Other past medical history reviewed. Medication list reviewed. Surgical history reviewed and updated. Reported: No recent change in medical history.

Screening findings and result

Previous breast digital diagnostic x-ray (02/22, 01/23, 02/23, 03/10/2019, 2/11/2020)

These previous imaging studies were provided for the purpose of comparison in the current study. During the examination, a lump was detected in the right breast, specifically a 1 cm oval mass at 7 o'clock anterior depth. Concerned about this finding, the patient underwent a right breast removal on April 19, 2023 suspecting for malignancy. Following the surgery, a bilateral tomosynthesis was performed on the day after Memorial Day in 2024 to evaluate the remaining breast tissue. It revealed an oval-shaped fibroid measuring approximately 9 mm in the left breast. To address this finding, fibroid removal surgery was scheduled for 6/6/2024. The patient initially agreed to the surgery, which was set for the following day. However, on the day of the scheduled procedure, the patient called and cancelled, expressing feelings of depression and hopelessness and stating her reluctance to lose her breast. She was advised to seek psychological counselling and reschedule the surgery as soon as possible. The patient went for double breast removal on August 25, 2023, and the fibroid and malignancy areas are removed. The patient is advised to follow up with a routine mammogram check. A mammogram was again ordered for September 25, 2022, but the patient did not attend the appointment due to personal reasons.

Following the stool DNA test, the patient received a positive result indicating the presence of genetic abnormalities and bio markers associated with colorectal

al cancer. Based on the positive stool DNA test result, the patient's healthcare provider performed A large bowel endoscopy was performed on 11/12/2023, ensuring a comprehensive evaluation of the patient's colon and

rectum. Another procedure was carried out on 12/23/2019 in accordance with standard medical protocols, utilizing a flexible tube with a camera to visualize the entire colon and rectum during the examination. Abnormal tissues and polyps were identified and appropriate measures were taken. In 2024 she received a diagnosis of colorectal cancer after a positive fecal occult blood test, leading to a Screening and polypectomy. During the colorectal Screening using flexible tool, multiple polyps were discovered, with one exhibiting high-grade dysplasia. She is currently under surveillance for potential recurrence. Patient undergoes for surgical procedure on in which the entire colon (large intestine) is removed. This procedure is performed due to the medical conditions that severely affect the colon, In this case of advanced colorectal cancer, so surgery is performed to remove the cancerous tissue and prevent further spread on 9/2/2023. Patient is scheduled for colonoscopy on 10/01/2014.

Family History: The family history reveals that the father passed away at the age of 85, and there were no significant medical conditions reported in the family. There is no history of early deaths among family members. However, there is a family medical history of various conditions, including coronary artery disease, acute myocardial infarction, systemic hypertension, hyperlipidemia, diabetes mellitus, and stroke syndrome. Specifically, the paternal side has a history of stroke or cerebrovascular accidents. The mother, who lived until the age of 90, was diagnosed with Alzheimer's disease.

Additionally, a sister in the family passed away at the age of 70, and no significant family medical history was reported in her case. It is important to consider this family history when assessing individual health risks.

Social History

Medical: Currently wearing eyeglasses for reading. Social history reviewed and social history unchanged. Personal: The patient has experienced recent emotional stress and also reports chronic emotional stress. Caffeine Use: The patient consumes 2-3 cups of coffee daily. Current Smoking Status: The patient is reported starting smoking in January 2000. She smokes an average of 10 cigarettes per day and has attempted to quit smoking twice without success. She states that she does not currently have any smoking - related illness.

Alcohol: The patient reports alcohol consumption primarily on weekends for the past five years. She estimates consuming 2-3 drinks, mainly beer with occasional wine or cocktails. The patient denies any history of binge drinking or alcohol-related incidents and states that she usually stays within the recommended weekly limits for alcohol consumption.

Drug Use: Not using drugs. Housing and Economic Circumstances: Does not live alone; grandson and family reside with the patient. The living environment is satisfactory. Work: Work history cashier/caregiver.

Marital: Marital history widowed. No diagnosis of opioid use. Informed healthcare decisions.

Review of System

Systemic: No fever, chills, or night sweats. Cardiovascular: No chest pain or discomfort. Pulmonary: No dyspnea or cough. Gastrointestinal: No nausea, vomiting, diarrhea, or constipation. Social: No recent travel to a foreign country.

Physical findings

Vitals taken

BP was recorded as 156/90 mmHg, followed by 138/84 mmHg (as reported by the patient) recorded on August 3, 2024. On August 8, 2023, the patient was brought to the ER with a blood pressure reading of 140/90 mmHg in the left arm. Two days after the ER visit, the patient reported that their blood pressure at home had been consistently ranging from 144-151/83-88 mmHg. As a result, the patient was advised to consult with their healthcare provider for blood pressure monitoring on August 20, 2024, but they later called and cancelled the appointment. The day after canceling, the patient visited the office and recorded a blood pressure of 140/90 mmHg using a non-digital blood pressure machine. Pulse Rate: 94 bpm.

Pulse Rhythm: Regular.

Respiration Rate :20 per min

Height: 62.5 in. Weight: 141 lbs.

Body Surface Area: 1.66m².

SPO₂: 94%. O₂ Device: None (Room Air). FiO₂: 21%.

The HbA_{1c} routine test was performed on February 10, 2023, and the result was >9%. However, after two days, the provider performed another HbA_{1c} test (CPT 83036), and the result obtained was 6.5%. On Feb-15-2023 the value observed is 6.6-6.9

Retinal Diagnostic Report

Southdale Eye clinic

Patient information

Name: Sarah Johnson
DOB:12/05/1971
Medical record number:
098-67-4567 Exam
Date:11/21/2024
Gender: Female
Referring Physician:DR Sandhya

Ordering Provider:Dr.Jess L. Boysen, M.D.
Southdale Eye clinic
Performing Location:
6533 Drew Avenue South
Edina, MN 55435

Dx Date	Dx	Description	eye	Care plan
05/28/2023	E119	Type 2 diabetes mellitus without complications		Stressed importance of proper diet and exercise, Send letter to PCP , Stressed importance for compliance to medications.
05/28/2023	H35,033	Hypertensive retinopathy, bilateral		Stressed importance of proper diet. Educated patient on findings. Advised patient to continue taking all medications prescribed by PCP or specialist.
05/28/2023	H52.4	Presbyopia	OU	Advised patient to update spectacle lenses. Adaption to refractive correction expected. Glasses to be whom full-time.

Diagnosis:

Dr. Jess

05/28/2023

E119 - Type 2 diabetes mellitus with complications:

- Hypertensive retinopathy.

- H524 - Presbyopia.

Care Plan:

Stress the importance of proper diet and exercise.

Healthy retinal vasculature. No macular edema noted.

Send a letter to the Primary Care Physician (PCP).

Follow-up appointments recommended regularly.

IMPRESSION(S: (OD and OS)

Senescent cataracts;

Nuclear sclerotic cataracts;

Type II diabetes

Non proliferative diabetic retinopathy

Mild non proliferative diabetic retinopathy. Without Macular Edema And Diabetes Type 2

No macular edema

Disposition:

Patient is pleasant and sociable.

Orientation: Patient is fully alert to time, place, and person.

Diagnosis of hypertensive retinopathy is positive.

Patient's adaptation to refractive correction is expected.

POSTERIOR SEGMENT:

Vitreous body clear for age and fully attached.

Nerve head well perfused, with good rim tissue.

Healthy macula with no edema or degenerative pigmentation.

Healthy peripheral retinal structures and vasculature.

No drusen, exudate, hemorrhages, or evidence of retinopathy unless otherwise noted below.

RETINA - VASCULAR:

DfE performed. No drusen, exudate, hemorrhages, or evidence of retinopathy unless otherwise noted below.

Unilateral Right eye enucleation performed on 7/28/2024

General Appearance

Nose:

General/bilateral:

Suppleness: Neck demonstrated no decrease in suppleness.

Ears:

General/bilateral:

Tympanic Membrane: Both tympanic membranes were normal.

Discharge: No nasal discharge seen.

Lymph Nodes:

Cervical Cervical lymph nodes were not enlarged.

Pharynx:

Oropharynx: Normal.

Lungs:

Clear to auscultation.

Cardiovascular:

Heart Rate and Rhythm: Normal. Murmurs: No murmurs were heard! Edema: Not present.

Back:

Tenderness on palpation. Muscle spasm.

Skin:

Mucous membranes were normal.

LABS Done

September 25, 2024, a urine protein test was conducted to assess protein levels and kidney function, resulting in 10 mg/dL (within the reference range of 0-20 mg/dL). An estimated glomerular filtration rate (eGFR) test was also performed, with an ICD-10, revealing a previous eGFR of 70 mL/min/1.73m² and a current eGFR of 75 mL/min/1.73m².

MLAB :19.1

U Creat:167.80

MALBCREA:11.4

THERAPY

Clinical summary provided to the patient.

GYN History:

The patient presented with heavy and prolonged menstrual bleeding (menorrhagia) and persistent pelvic pain, which significantly affected her quality of life. After thorough evaluation, the patient underwent cervical endocervical cytology, performed on 1/12/2021. It was determined that the patient had the following medical conditions that necessitated the performance of an **abdominal surgery of uterus with surrounding** after **three days**: - Uterine Fibroids: Multiple non-cancerous growths (fibroids) in the uterus causing enlargement and excessive bleeding. - Adenomyosis: A condition where the tissue lining the uterus (endometrium) grows into the muscular wall of the uterus, leading to pain and heavy bleeding. Chronic Pelvic Inflammatory Disease: Persistent inflammation and infection of the pelvic organs, contributing to pelvic pain and discomfort. Additionally, since 2022, she has been dealing with polycystic ovarian syndrome and has initiated treatment with metformin to address her symptoms. Incision: A transverse incision was made in the lower abdomen to access the pelvic organs. **Uterine Dissection**: The surgeon carefully dissected the uterus from surrounding structures, including ligaments and blood vessels. Bilateral Salpingo-Oophorectomy: During the procedure, the surgical team observed the following: - **Multiple Uterine Fibroids**: Several fibroids of varying sizes were present in the uterine wall. - **Enlarged Uterus**: The uterus was enlarged due to the presence of fibroids and adenomyosis. - **Adhesions**: Pelvic adhesions were observed, which could contribute to the patient's pelvic pain. In addition to the uterus, both fallopian tubes and ovaries were removed to prevent future complications and reduce the risk of ovarian cancer. ****Closure: **** The incision was meticulously closed using sutures. Following the **abdominal surgery on the uterus**, the patient was closely monitored in the recovery room for the appropriate period. Vital signs, pain levels, and surgical site were regularly assessed.

Other History: The individual, born on 12/05/1971, has a recorded history of immunizations. They received the vaccine on **12/02/1984**, providing protection against Diphtheria.

However, **three days** after this, the member experienced an anaphylactic reaction, emphasizing the importance of caution during future vaccinations. A **cervariz dose 1** was administered on 2/06/2019 to maintain immunity against the aforementioned diseases. Subsequently, Second dose MCv4 scheduled and received after **six months on same day**. Subsequently, on approximately 2/03/2024, they received the seasonal flu vaccine for influenza protection. The individual had a **scheduled** HPV vaccine appointment on 10/05/2023, and the status indicates that the appointment was **canceled**.

Metro Healthcare Radiology,
Board Certified Fellowship,
Breast Imaging
Electronically Signed on: 08-03-2024
21:38:50 ELLA,LORI :A0240072:628600

emily

Follow up after Emergency visit department for people with multiple high risk chronic conditions(FMC)

- **Primary Care Physician (PCP): Dr. Sarah Johnson**
- **Emergency Department Visit: 2024-07-15**
- **Chronic Conditions: Hypertension, Type 2 Diabetes, Asthma, Obesity, Hyperlipidemia**

1. Emergency Department Visit:

- **Date of Service (DOS): 2024-07-15**
- **Reason for Visit: Severe shortness of breath, chest tightness, and dizziness.**
- **Clinical Summary:**

Chief Complaint: Severe shortness of breath, persistent chest tightness, dizziness for the last 6 hours.

Vitals on Admission:

- **Blood Pressure: 170/90 mmHg**
- **Heart Rate: 110 bpm**
- **Respiratory Rate: 22 bpm**
- **SpO2: 92% on room air**

Past Medical History: Hypertension, Type 2 Diabetes, Asthma (moderate persistent), Obesity, Hyperlipidemia.

Medications:

- **Metformin 500 mg daily**
- **Lisinopril 10 mg daily**
- **Albuterol inhaler PRN**
- **Atorvastatin 20 mg daily**
- **Insulin as needed**

Physical Examination:

- **Respiratory: Wheezing, prolonged expiration.**
- **Cardiovascular: Tachycardia, no murmurs.**
- **Neurological: Alert, oriented, no focal deficits.**

Diagnostics:

- **Chest X-ray: No acute findings.**
- **EKG: Sinus tachycardia.**

- **Blood Gas:** Mild metabolic acidosis.
- **Laboratory:** Elevated glucose levels, elevated blood pressure, slightly raised cholesterol.

Diagnosis:

- **Asthma exacerbation**
- **Hypertensive crisis**
- **Poorly controlled Type 2 Diabetes**

Treatment Plan:

- **Administered nebulized albuterol and ipratropium.**
- **Intravenous fluids and antihypertensive medications (labetalol).**
- **Blood glucose levels monitored, insulin administered.**
- **Advised to follow up with primary care provider (PCP) for medication adjustment.**

2. Notification of Inpatient Admission (2 Days after Admission):

- **Date of Admission: 2024-07-15**
- **Date of Service (DOS): 2024-07-15**

Received Notification of Admission: The admission notification was documented on the day of admission, 2024-07-15, through the automated system alert from the ED to the health plan and PCP.

Documentation: The notification of inpatient admission was recorded in the EMR system, confirming receipt by the PCP and health plan.

Date of Communication (2 days after admission): 2024-07-17

Provider Communication: ED physician communicated the admission to Dr. Sarah Johnson (PCP) via EMR.

Health Plan Notification: The health plan received an automated alert regarding the patient's admission, with a summary of Sarah's chronic conditions, treatment administered, and follow-up care needs. This was documented within the health information exchange system.

3. Receipt of Discharge Information (2 Days after Discharge):

- **Date of Discharge: 2024-07-17**
- **Date of Service (DOS): 2024-07-17**

Receipt of Discharge Information:

- **Day of Discharge: 2024-07-17**
The discharge summary was sent from the hospital's EMR system to the health plan and PCP, documenting the treatment provided, discharge plan, and follow-up care instructions.
- **2 Days After Discharge: 2024-07-19**
The discharge information was fully received and logged by Dr. Sarah Johnson's

office and the health plan's system. This included details of the patient's discharge medication regimen, follow-up instructions, and changes to existing therapies.

4. Patient Engagement After Inpatient Discharge (30 Days of Discharge):

- **Date of Discharge: 2024-07-17**
- **Date of Service (DOS): 2024-07-17**

Engagement within 30 Days of Discharge:

- **Date of Engagement: 2024-07-25 (within 30 days of discharge)**
Sarah received a follow-up phone call from Dr. Sarah Johnson's office to discuss her recovery, reassess her medication regimen, and ensure she was adhering to the prescribed changes. The phone call included a reminder to monitor her blood pressure and blood glucose levels, and to attend the upcoming follow-up appointment.
Engagement Documentation: This was documented in the EMR system, confirming that Sarah engaged with her healthcare provider post-discharge within the required 30-day window.

5. Medication Reconciliation Post-Discharge (30 Days of Discharge):

- **Date of Discharge: 2024-07-17**
- **Date of Service (DOS): 2024-07-17**

Medication Reconciliation:

- **Date of Medication Reconciliation: 2024-07-19 (within 2 days of discharge)**
Medication reconciliation was performed by Dr. Sarah Johnson's office staff, ensuring that all new prescriptions and adjustments made during the hospital stay were accurately recorded and that Sarah was provided with an updated medication list.

Medications Adjusted:

- Increased Lisinopril dose for blood pressure control.
- Adjusted insulin dosage based on recent glucose levels.
- New inhaler prescription for asthma management.
- Discontinued temporary antihypertensive IV therapy from the hospital.

The medication reconciliation was completed, documented, and sent to Sarah's pharmacy and health plan.

Follow-up for Medication Reconciliation:

- **Date of Follow-up Check: 2024-07-25 (within 30 days of discharge)**
A phone call was made to confirm that Sarah was able to pick up her medications and had no issues with the new prescriptions. Any concerns were addressed, and an updated medication list was provided.

6. Future Follow-up Care & Monitoring:

- **Next Follow-up Appointment:**
 - **Scheduled for: 2024-07-22 (within 7 days of discharge)**
The follow-up visit will assess Sarah's condition, review her medication regimen, and evaluate her chronic disease management.
- **Ongoing Provider Communication:** Dr. Sarah Johnson will continue to monitor Sarah's health through regular visits and follow-up calls, ensuring her chronic conditions remain stable and her medication regimen is optimized.

Patient Medical Record:

Primary Obstetrician: Dr. Lisa Moore

PCP: Dr. Amanda Green

Emergency Contact: Mark Thompson (Husband)

Date of Visit: 01/09/2024

Obstetric History:

- **Gestational Age at Visit:** 13 weeks and 1 day (13w1d)
- **Gravida/Para Status:** G2P1 (2 pregnancies, 1 live birth)
- **Estimated Due Date (EDD):** 07/20/2024
- **Previous Pregnancy Outcome:** Full-term vaginal delivery, healthy baby.
- **Medications:**
 - **Dopetone 5 mg:** For mood stabilization (or another indication).
 - **Prenatal Vitamins (Folic Acid 400 mcg)**
 - **Calcium Supplement (500 mg daily)**
 - **Iron Supplement (325 mg daily)**
 - **Fenugreek 500 mg TID (for lactation support after delivery).**
- **Prenatal Visit (13w1d):**

- **Vitals:**
 - **Blood Pressure:** 118/78 mmHg
 - **Heart Rate:** 78 bpm
 - **Temperature:** 98.2°F
 - **Respiratory Rate:** 16 breaths per minute
 - **Weight:** 135 lbs (increase from baseline weight)
- **Physical Exam:**

- **Abdomen:** Non-tender, no distension, uterus palpated at 13 cm above pubic symphysis, consistent with gestational age.
- **Breast Exam:** Normal, no tenderness or signs of mastitis.
- **Pelvic Exam:** Normal, no abnormalities observed.
- **Ultrasound (13 Weeks):**
 - **Fetal Heartbeat:** Detected strong fetal heart tones at 150 bpm.
 - **Crown-Rump Length:** 6.5 cm (consistent with 13 weeks gestation).
 - **Amniotic Fluid:** Normal.
 - **Placenta:** Located on the anterior wall, no abnormalities.
- **Screening and Tests:**
 - **NT Scan (Nuchal Translucency):** Scheduled for 13w3d.
 - **Blood Tests:** Routine first trimester screen for genetic conditions.
 - **Results Expected:** In the next 2 weeks.
 - **Fetal Movement:** Not yet palpable (expected after 18–20 weeks).
 - **Edema:** None noted at this time.

Review of Prenatal Care:

- **Date of Prenatal Visit:** 01/09/2024 (13w1d)
- **Evidence:** Standardized prenatal flow sheet, LMP, EDD confirmed, and gestational age assessed.
- **Counseling:**
 - **Prenatal Risk Counseling:** Discussion around lifestyle choices (nutrition, exercise), potential complications, and signs to watch for (e.g., severe bleeding, high blood pressure).
 - **Genetic Screening:** Offered, with NT scan scheduled for 13w3d.
- **Physical Examination:**
 - **Auscultation of Fetal Heart Tone:** Heard with a Doppler, normal at 150 bpm.
 - **Pelvic Exam and Obstetrical Observation:** Normal; no abnormalities observed during the pelvic exam.
 - **Fundal Height Measurement:** Uterus palpated at 13 cm, consistent with gestational age.
 - **Blood Typing:** Rh compatibility testing completed, no evidence of Rh incompatibility.
- **Screening Procedures Performed:**

- **Blood Screening Test (TORCH Panel and Rubella Test):** Screening for infectious diseases (e.g., rubella immunity, toxoplasmosis) ordered.
- **Ultrasound:** Confirmed normal fetal heart tone, placental location, and amniotic fluid levels.
- **Nuchal Translucency Scan (NT Scan):** Scheduled for 13w3d.

Assessment and Plan:

- **Diagnosis:**
 - **13 weeks pregnant with a normal, uncomplicated pregnancy.**
 - **Gravida 2, Para 1 with no complications noted.**
 - **On Dopestone for mood stabilization (or another indication), well-tolerated with no adverse reactions.**
- **Management and Follow-Up:**
 - **Next Prenatal Visit: 16 weeks gestation (02/2024) for second trimester screening and ultrasound.**
 - **Nuchal Translucency Screening: Scheduled for 13w3d.**
 - **Blood Tests: Routine first trimester screening (results expected soon).**

Active Management Plan:

- **Mental Health Screening:**
 - **Depression and Anxiety Screening: Scheduled for future visits (including postpartum screening).**
 - **Counseling for Emotional Well-being: Discussed stress management techniques.**
- **Fetal Monitoring:**
 - **Routine monitoring of fetal heart tones, growth, and movement.**
 - **Follow-up for any symptoms of preterm labor or complications.**
- **Medications:**
 - **Continue current medications (Dopestone, Prenatal vitamins, Calcium, and Iron).**
 - **Fenugreek for lactation support to be used after delivery.**

Postpartum Care Plan (for after delivery):

- **Postpartum Visit: On or between 7 and 84 days after delivery.**
 - **Scheduled Postpartum Visit Date: 09/25/2024 (2 weeks postpartum).**
- **Postpartum Counseling:**
 - **Breastfeeding Education: Encourage continued breastfeeding; discuss pumping options.**

- **Depression Screening:** Postpartum depression screening (EPDS) will be conducted during follow-up.
- **Follow-Up Plan:**
 - **Next Visit:** 16 weeks gestation (February 2024).
 - **Ultrasound and Genetic Screenings:** Follow-up on first trimester screening and NT scan results.
 - **Regular Mental Health Screenings:** Monitor emotional well-being, particularly in the postpartum period.
 - **Postpartum Visit:** Scheduled for 09/25/2024 (between 7–84 days post-delivery). This visit will include:
 - Vitals check
 - Breastfeeding support and counseling
 - Screenings for depression and anxiety (EPDS)
- **Discharge Medications (If applicable after Delivery):**
 - **Dopetone:** Continue as needed.
 - **Prenatal Vitamins (Folic Acid, Calcium, Iron):** Continue until 6 weeks postpartum.
 - **Fenugreek (for breastfeeding):** 500 mg TID (post-delivery).

Provider's Notes:

- Patient continues to progress well in her pregnancy with no complications or concerns. Dopetone is being used effectively, and fetal growth appears normal with healthy development. The NT scan and first trimester screening will be completed as planned. The patient is scheduled for follow-up care, with further education on mental health and postpartum support.

Attachments and Additional Notes:

- Prenatal Flow Sheet and Screening Results (for internal use).
- Fetal Ultrasound Report (attached).
- Nuchal Translucency Scan Report (upcoming).

Osteoporosis Management Report (OMW)

Primary Care Provider: Dr. Elaine Richardson.

Orthopedic Specialist: Dr. John Smith

Date of Visit: 12/01/2024

Medical History:

Patient who was diagnosed with osteoporosis after a recent left femur fracture on June 25, 2024. She has a medical history that includes osteopenia diagnosed four years ago following a routine bone

density test, and a family history of osteoporosis (both her mother and aunt had fractures). Sarah has been postmenopausal for two years, and she recently fell at home, resulting in the fracture. She reported pain during movement and limited mobility.

Current Medications:

Medication Name	Generic Name	Brand Name	Dosage	Route	Frequency	Prescription Dispensed Date
Alendronate Sodium	Alendronate	Fosamax	70 mg	Oral	Once a week	08/01/2024
Calcium Carbonate	Calcium Carbonate	Caltrate	600 mg	Oral	Twice daily	08/05/2024
Vitamin D3	Cholecalciferol	Nature Made	1000 IU	Oral	Once daily	08/05/2024
Teriparatide	Teriparatide	Forteo	20 mcg	Subcutaneous	Once daily for 6 months	09/15/2024

Bone Density Test Results:

Test Name	Test Date	Findings
Dual-Energy X-Ray Absorptiometry (DXA)	07/10/2024	Lumbar Spine T-Score: -2.8 (severe osteoporosis), Total Hip T-Score: -2.5
CT Bone Mineral Density Study	07/12/2024	Spinal BMD: 0.850 g/cm ² (moderate osteoporosis)
Ultrasound Bone Density Study	07/08/2024	Heel Bone Density: Low bone mineral density

Assessment and Management Plan:

- **Diagnosis:** Osteoporosis with recent left femur fracture (06/2024).
- **Risk Factors:** Post-menopausal, family history of osteoporosis, low bone density (confirmed by DXA and CT scans), fall-related fracture.
- **Fracture Healing:** Healing as expected, no complications observed during follow-up visits.

Medication Management:

Sarah has been placed on a medication regimen for managing her osteoporosis:

- **Alendronate (Fosamax):** 70 mg weekly.
- **Calcium Carbonate (Caltrate):** 600 mg twice daily.
- **Vitamin D3 (Nature Made):** 1000 IU daily.
- **Teriparatide (Forteo):** 20 mcg once daily for 6 months.

The medications prescribed are intended to improve bone density, prevent further fractures, and aid in healing. Patient was also advised to follow a balanced diet rich in calcium and vitamin D and engage in weight-bearing exercises like walking or swimming.

Follow-Up Plan:

- **Bone Density Test:** Recheck DXA and CT scans in 6 months to assess the effectiveness of treatment and monitor fracture healing.

- **Next Visit: 06/2024** for follow-up on fracture healing and to monitor the efficacy of the medication regimen.

Consolidated Summary:

This **comprehensive patient case** for **patient** involves osteoporosis management following a **left femur fracture**. The **medication regimen** was prescribed and dispensed within **6 months of the fracture**, as per the clinical guidelines for managing osteoporosis after a fracture. The medications, including **bisphosphonates** and **Teriparatide**, were initiated to help improve bone density and prevent future fractures. Regular **bone density testing** was performed, including **DXA**, **CT**, and **ultrasound** studies, which confirmed severe osteoporosis in Sarah’s lumbar and hip regions. Follow-up visits and bone density reassessments are scheduled to ensure the effectiveness of the treatment plan and continued healing.

Medication Reconciliation Post-Discharge Report

Patient was discharged on January 1, 2024, following treatment for an acute myocardial infarction.

Discharge Medications

The following medications were prescribed at the time of discharge:

Medication	Dosage	Frequency	Indication	Notes
Atorvastatin	40 mg	Once daily (evening)	Cholesterol management	Taken to reduce LDL cholesterol levels.
Metoprolol	50 mg	Twice daily	Blood pressure and heart rate	Helps reduce heart workload and prevent arrhythmias.
Aspirin	81 mg	Once daily (morning)	Antiplatelet therapy	Prevents clot formation.
Clopidogrel	75 mg	Once daily (morning)	Prevention of blood clots	Reduces risk of recurrent heart attacks.

Follow-Up Visit

Date	Details
Visit Date	January 15, 2024
Physician	Dr. Emily Carter (Cardiologist)
Symptoms Reported	Occasional dizziness in the afternoon.
Vital Signs	Blood pressure: 120/75 mmHg, Heart rate: 65 bpm.

Actions Taken:

Metoprolol Adjustment: Dosage reduced from 50 mg to 25 mg twice daily due to reported dizziness.

Updated Prescription (Dispensed on January 16, 2024)

Medication	Adjusted Dosage	Quantity Dispensed	Frequency	Indication
Atorvastatin	40 mg	30 tablets	Once daily (evening)	Cholesterol management
Metoprolol	25 mg	60 tablets	Twice daily	Blood pressure and heart rate
Aspirin	81 mg	30 tablets	Once daily (morning)	Antiplatelet therapy
Clopidogrel	75 mg	30 tablets	Once daily (morning)	Prevention of blood clots

Medication Reconciliation

- Compliance: All medications were dispensed within the required 31-day window post-discharge.
- Changes: The Metoprolol dose was reduced due to dizziness, aligning with the patient’s condition and physician’s recommendation.

Patient Education

The patient was educated on the importance of adhering to the updated dosage regimen. He was advised to:

1. Monitor for any further side effects, such as persistent dizziness.
2. Take medications at the recommended times consistently.
3. Contact the healthcare provider if new symptoms arise.

Next Follow-Up

- Scheduled Date: February 15, 2024
- Purpose: Assess the effectiveness of the medication adjustment and monitor overall recovery progress.